

--SUMMARY--

Decision No. 1838/12

03-Oct-2012

T.Mitchinson

- Subsequent incidents (outside work)

No Summary Available

7 Pages

References: Act Citation

- WCA

Other Case Reference

- [w4712n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1838/12

BEFORE:

T. Mitchinson: Vice-Chair

HEARING:

September 27, 2012 at Toronto
Oral

DATE OF DECISION:

October 3, 2012

NEUTRAL CITATION:

2012 ONWSIAT 2173

DECISION(S) UNDER APPEAL: WSIB ARO decision dated December 3, 2008

APPEARANCES:

For the worker:

R. Sequin, Office of the Worker Adviser

For the employer:

Closed

Interpreter:

N/A

REASONS

(i) Introduction

[1] This appeal was heard in Toronto on September 27, 2012. The worker appeals the December 3, 2008 decision of Appeals Resolution Officer (ARO) F. Pansino. The ARO denied the worker entitlement for the left shoulder as a secondary injury stemming from his compensable left knee injury.

[2] The worker attended the hearing and was represented by Raymond Sequin from the Office of the Worker Advisor. The employer is closed. The worker testified and Mr. Sequin made submissions on behalf of his client.

(ii) Applicable law

[3] The worker's compensable left knee injury occurred on August 30, 1985. Therefore, the pre-1989 *Workers' Compensation Act* (the Act) applies.

(iii) The issue on appeal

[4] The only issue in this appeal is whether the worker has entitlement for the left shoulder as a secondary injury stemming from his compensable left knee injury claim.

(iv) Background

[5] The worker suffered a left knee fracture and rupture of the popliteal artery on August 30, 1985 while performing regular job duties as a factory worker with the accident employer, a steel fabrication company. He reported the injury to his employer and filed a claim with the Workers' Compensation Board (the Board). The claim was accepted, and the worker received temporary total disability benefits to April 1986 when he returned to work.

[6] The Claims Adjudicator assigned to the worker's file concluded that the worker had sustained a permanent impairment of the left knee, and he was granted a 12.5% permanent disability (PD) award in April 1998.

[7] In November 2005, the worker underwent a total left knee replacement, after which his PD award was raised to 25%.

[8] In October 2001, the worker fell and dislocated his left shoulder. He also experienced other falls, including one in April 2003.

[9] In 2006, the worker sought to expand his left knee claim to include the left shoulder injuries. The Adjudicator referred the issue for an investigation to obtain additional information concerning the relationship between the left shoulder condition and the worker's compensable injury.

[10] After reviewing the results of the investigation and consulting with a Board Medical Advisor, the Adjudicator denied entitlement for the left shoulder on the basis that there was insufficient evidence to establish a causal relationship between the worker's knee condition and any left shoulder injuries.

[11] The Adjudicator's decision was upheld by ARO Pansino in his December 3, 2008 ruling.

(v) The worker's position

[12] The worker testified that his left knee injury left him unstable, and the knee gave out on a number of occasions causing him to fall.

[13] The worker recalled one such occasion in October 2001 when he fell on the front sidewalk to his home, landing on his left shoulder on the concrete. He could not recall why he was leaving the home that day, but assumed that he was probably driving his wife on an errand, which was a common occurrence. The worker did recall that his daughter drove him to the emergency department of the local hospital where, after waiting for a long time, his dislocated shoulder was put back into the socket and he was sent home with a sling. According to the worker, within the next few days his shoulder dislocated again spontaneously, and he returned to the emergency department to have it put back in place.

[14] Although the worker was clear in his testimony that the fall was caused by his left knee giving out, he could not explain why there was no reference to this in any medical reporting at the time, nor why the reports make specific reference to the fall having resulted from slipping on wet grass.

[15] As far as the second claimed shoulder injury in April 2003 is concerned, the worker had no memory of the fall or the circumstances that caused it, nor could he recall explaining the injury on the basis of slipping on sand that he was putting on his driveway.

[16] The worker also had no recollection of speaking with the Board investigator who met with him in April 2006 to discuss the two claimed shoulder injuries.

[17] Mr. Sequin accepts that medical reporting in the context of the worker's October 2001 and April 2003 shoulder injuries do not mention his left knee giving out as the cause. However, in his view, the Board investigator's report fills this evidentiary gap, at least with respect to the October 2001 injury. In that report the worker recalled that the grass was wet on the day in question, but that it was his left knee and not the wet grass that caused him to fall.

[18] Mr. Sequin also relies on statements made by the Board Medical Advisor, who reviewed the claim on several occasions, that the worker's shoulder injury was compatible with and probably caused by the left knee having given out.

[19] In Mr. Sequin's view, the only reasonable explanation for the worker falling is that his left knee gave out due to his significant and serious compensable injury. He points out that the worker, for whatever reason, did not advise the Board of the 2001 and 2003 injuries until 2006, which could explain why the relationship between the knee injury and the subsequent shoulder problems was not more thoroughly documented.

(vi) Analysis and findings

[20] Board *Operational Policy Manual* (OPM) Document No. 15-05-01 allows for entitlement to secondary injuries resulting from compensable workplace accidents. It states:

Workers sustaining secondary conditions that are causally linked to the work-related injury will derive benefits to compensate for the further aggravation of the work-related impairment or for new injuries.

...

Entitlement for any secondary condition is accepted when it is established that a causal link exists between it and the work-related injury. The development of a left knee disability/impairment due to an increased dependency following a work-related injury to the right knee, is an example.

[21] Accordingly, the issue here is whether the worker's left shoulder condition, which is not in dispute, was caused by his compensable workplace accident involving his left knee.

[22] The Case Record contains a number of medical reports from 2001 following the first fall and subsequent treatments, as well as other records from 2003 when the shoulder injury was treated after the second fall. Having carefully reviewed these records, I find that only two of them make any reference to the cause of the falls.

[23] The first is an October 23, 2001 report from orthopaedic surgeon, Dr. Rittenhouse, who undertook a closed reduction of the left shoulder to correct the injury. Dr. Rittenhouse states:

One week ago on October 13 he [the worker] was outside, and he slipped on a wet area and fell injuring his left shoulder.

[24] The second record is a March 13, 2003 report from a different orthopaedic surgeon, Dr. Ansari, who performed another closed reduction after the second fall. He states:

Patient was seen with a history that last Thursday he was putting some sand on his driveway and he slipped and landed on the left shoulder and he said he dislocated it. Then he also hurt his knee.

[25] Neither treating physician makes any mention of the worker's left knee having given out at the time of the falls.

[26] This is not a situation where there is no documentary evidence pointing to a cause of injury. Rather, in both situations the treating physicians explain the circumstances that led to the falls based on factual details contemporaneously provided by the worker. In my view, had the falls been caused by the worker's left knee giving out, it is reasonable to assume that he would have explained that to the doctors at the time. It is obvious from the content of the reports that both doctors asked the worker to explain the circumstances, and there is no reason to doubt that they recorded his answers accurately in their reports.

[27] Mr. Sequin argues that the Board Medical Advisor offered the opinion that the worker's shoulder injuries were compatible with his left knee giving way, and that this was the probable cause of the left shoulder problems. That is not completely accurate.

[28] In his first note dated January 18, 2006, Medical Advisor, Dr. Dorcas, states that "a fall would be compatible with the knee injury", but goes on to point out that "there is no information on file to determine the condition of the left shoulder".

[29] In his next note of April 6, 2006, Dr. Dorcas repeats that a fall would be compatible with the left knee injury, and recommends that an investigation be initiated "to determine the relationship of a left shoulder problem to falls related to the left knee giving out".

[30] Dr. Dorcas' final report, dated May 23, 2006, sums up his position as follows:

The medical reports do not mention that the falls were caused by the left knee giving out. In memo #37 [the January 18, 2006 note] I stated that a fall would be compatible with the knee injury, due to an unstable knee with a tendency of giving way. However I don't know if any of the falls resulting in the shoulder problems were caused by the knee giving out.

[31] It is clear from these reports that, although Dr. Dorcas identifies a potential linkage between the worker's compensable left knee injury and the subsequent shoulder injuries, he does not conclude, as suggested by Mr. Sequin, that there is a probable causal connection. Entitlement for a secondary injury claim requires evidence of a causal linkage, and I find that the medical evidence available up to the time of Dr. Dorcas' final review falls short of the required standard.

[32] The only other relevant medical reports in this case are a March 16, 2006 letter to the Board from orthopaedic surgeon, Dr. Robert, who replaced the worker's left knee in November 2005; and an August 14, 2008 letter from Dr. Ansari to the worker's former representative concerning the worker's shoulder claim.

[33] In his report, Dr. Robert states that:

Approximately three years ago, his [the worker's] knee was giving out, causing him to fall on three occasions.

...

I feel that this [the worker's shoulder injuries] is certainly related to the previous knee problem, as he has fallen on three occasions, requiring surgical intervention.

[34] Dr. Ansari was asked by the former representative to offer an opinion as to whether the 2001 and 2003 shoulder injuries were related to the left knee injury. He states:

Is this related to his knee injuries? The only thing I can say is that the left shoulder was dislocated in October 2001 is that he fell and hurt himself and I didn't see him then. So there is a possibility that the knee gave out on him but I do not know, as I did not see him then. The first time I saw him was on March 18, 2003. At that time he did fall in his driveway, now did he slip or did his knee give out on him because there is no doubt that he does have bad knees and is related to compensable injury of a fractured tibial plateau. So the probability is that the knee gave out and he fell.

...

The next question is if the shoulder injury was caused by the knee giving out. I would feel that the first fall he had in October 2001, which is the fall that started the entire process of him having damage to the shoulder joint. If that fall is related to his knee giving out on him and falling, then of course it would be related, but I did not see him at that time. If one can confirm that his knee gave out on him and that is why he fell in October 2001 and resulted in the dislocation of the shoulder, then the dislocation of the shoulder would be directly related to the fall in October 2003 [should be 2001]. As mentioned, the only time I saw him was March 18, 2003.

[35] I give minimal weight to these two medical reports.

[36] In the case of Dr. Robert, he did not begin treating the worker until 2005, and has no direct knowledge of the circumstances leading to either the 2001 or the 2003 fall. He bases his opinion on information provided by the worker in 2005, which is not consistent with information provided earlier to the two treating physicians, and I find that Dr. Robert's conclusions linking the shoulder and knee injuries to be speculative and insufficiently factually grounded.

[37] As far as Dr. Ansari is concerned, he too did not treat the worker in the context of his 2001 fall, and points out that confirmation that the worker's knee gave out in 2001 would have to come from someone else. He states that the 2003 fall was probably caused by the worker's knee giving out, but this is inconsistent with his report from March 2003 which states that the worker

fell while spreading sand on his driveway. I prefer the earlier version of events, given that his was prepared contemporaneously based on factual details provided by the worker at that time.

[38] Finally, Mr. Sequin submits that the worker's testimony at the hearing, as well as his comments made to the Board investigator in 2006, are sufficient to overcome the factual discrepancies from earlier reports concerning the circumstances leading to the falls.

[39] During the course of the investigation, the worker was asked to explain why the October 2001 report suggested that he slipped on wet grass. At that point the worker recalled telling the doctor that the grass was wet, but that he did not slip and that he fell as a result of his knee giving out. As far as the 2003 incident was concerned, the worker said it was possible that there was sand on the driveway at that time, but that he was not in the process of putting it there, and he fell because his knee gave out.

[40] At the hearing, the worker had difficulty recalling factual elements relating to his two shoulder injuries. For example, he did not recall any wet grass on the sidewalk in 2001; he had no recollection whatsoever of the circumstances concerning the March 2003 fall; nor could he recall meeting with the Board investigator in 2006. He was clear that the cause of the falls was his knee giving out, but could offer no explanation as to why there was no reference to this in any contemporaneous medical reporting.

[41] Although I accept that it has been several years since the 2001 and 2003 shoulder injuries, and that memories often fade with time, given the significant gaps in the worker's recollection of relevant events, I find that his testimony is not sufficient to overcome the evidentiary gaps and inconsistencies present in various medical records. In my view, the best evidence of the factual circumstances leading to the two falls is found in the October 23, 2001 report from Dr. Rittenhouse, and the March 13, 2003 report from Dr. Ansari. The two treating physicians described the circumstances based on information provided by the worker, and in each case the cause of the fall was attributed to events that had no relationship to the worker's compensable left knee injury.

[42] For all of these reasons, I find that the causal link necessary to establish entitlement for the left shoulder as a secondary condition relating to the worker's left knee claim under OPM Document No. 15-05-01 is not established on the evidence, and the worker's appeal is denied.

DISPOSITION

[43] The appeal is denied.

DATED: October 3, 2012

SIGNED: T. Mitchinson